

King Fahad Central Hospital

Document ID: SU-003-OS-01 | Parent Policy: SU-003 — Acute Ischemic Stroke Management — IV Thrombolysis and Mechanical Thrombectomy | Version: 1.0 | Controlled Copy

Acute Ischemic Stroke Admission Order Set - Stroke Unit (2026)

AHA/ASA 2026 guideline-aligned institutional order set supporting stroke quality and certification readiness

Patient Name: _____

DOB: _____ MRN: _____

Admitting Physician: _____

Allergies: _____

Date/Time of Admission: _____

Last Known Well (LKW): _____

Weight (kg, measured): _____

Code Status: ☐ Full Code ☐ DNR ☐ DNI ☐ Other: _____

Admitting Diagnosis: ☐ Acute Ischemic Stroke ☐ Acute Ischemic Stroke - etiology under evaluation ☐ Other: _____

Level of Care / Destination: ☐ Stroke Unit ☐ ICU/Critical Care ☐ Other: _____ Reason for ICU/escalation: _____

1. Stroke Quality / Certification Readiness Checklist

Verify completion/documentation during admission and before discharge according to the current institutional stroke-measure set and AHA/ASA-aligned practice.

- ☐ Dysphagia screen completed before any oral intake (food, fluids, or medication)
- ☐ NIH Stroke Scale (NIHSS) documented on arrival and repeated per protocol/clinical change
- ☐ Venous thromboembolism (VTE) prophylaxis addressed by end of hospital day 2
- ☐ Antithrombotic therapy administered/prescribed when indicated, with contraindication documented if omitted
- ☐ Anticoagulation plan documented for atrial fibrillation/flutter when indicated, including planned start timing if deferred
- ☐ Reperfusion eligibility and treatment documented (IV alteplase/tenecteplase and/or endovascular thrombectomy when indicated)
- ☐ Guideline-directed statin/lipid-lowering therapy ordered unless contraindicated; reason documented if not prescribed
- ☐ Stroke education provided to patient/caregiver (warning signs, risk factors, medications, follow-up)
- ☐ Assessed for rehabilitation services (PT/OT/Speech)
- ☐ Smoking cessation counseling/therapy offered if applicable
- ☐ Follow-up and secondary-prevention plan documented before discharge

2. Vital Signs & Neurological Monitoring

- ☐ Post-IV thrombolysis: neuro checks + vital signs q15min x 2 h, then q30min x 6 h, then q1h x 16 h, per institutional thrombolysis protocol
- ☐ If not thrombolysed: neuro checks + vital signs q1-2 h initially or per Stroke Unit/ICU protocol and clinical condition
- ☐ Continuous cardiac telemetry for at least 24-48 h unless a longer duration is clinically indicated
- ☐ Continuous pulse oximetry as clinically indicated; give supplemental O2 if needed to maintain SpO2 >94% (no routine oxygen in nonhypoxic patients)
- ☐ Strict intake and output when clinically indicated
- ☐ Notify physician immediately for new/worsening neurologic deficit, decreased LOC, severe headache, NIHSS increase ≥ 2 , temperature $>38.0^{\circ}\text{C}$, HR <50 or >120 , SpO2 $<94\%$, or BP outside the applicable target below

3. Activity & Positioning

- ☐ Head-of-bed and activity individualized to neurologic/hemodynamic status; avoid prolonged routine flat positioning unless specifically ordered for a documented indication
- ☐ Once clinically stable, position to reduce aspiration risk and promote safe mobilization; HOB elevation per swallowing/respiratory status and physician order
- ☐ Fall precautions
- ☐ Aspiration precautions
- ☐ Bedside swallow screen before first oral intake; formal Speech-Language Pathology evaluation if screen fails or examination is concerning
- ☐ PT/OT evaluation within 24-48 h of admission
- ☐ Intermittent pneumatic compression/SCDs while immobile unless contraindicated
- ☐ Out of bed to chair / mobilize as tolerated once medically stable - specify restrictions if any: _____

4. Diet

- ☐ NPO until dysphagia screen is completed
- ☐ If screen passed: ☐ Regular ☐ Cardiac/low-sodium ☐ Diabetic ☐ Texture-modified ☐ Other: _____
- ☐ If screen failed: NPO, SLP consult, consider IV fluids/alternate nutrition route
- ☐ Aspiration precautions if dysphagia confirmed

5. IV Fluids

- ☐ IV access: 1-2 peripheral IVs
- ☐ Isotonic IV fluid (e.g., 0.9% NaCl) at _____ mL/hr if needed; avoid hypotonic and routine dextrose-containing fluids acutely unless treating hypoglycemia
- ☐ Avoid hypotonic fluids in patients at risk of cerebral edema

6. Laboratory Studies

- ☐ CBC with differential
- ☐ Basic or comprehensive metabolic panel
- ☐ PT/INR, aPTT
- ☐ Fasting lipid panel
- ☐ Hemoglobin A1c
- ☐ Troponin
- ☐ Type and screen if clinically indicated (e.g., hemorrhagic-conversion/procedure risk)
- ☐ Urine or serum pregnancy test if applicable
- ☐ Additional tests only if clinically indicated (e.g., toxicology, hypercoagulable evaluation): _____

7. Imaging & Cardiac Workup

- ☐ Non-contrast head CT reviewed; intracranial hemorrhage excluded before ischemic-stroke antithrombotic orders
- ☐ CTA head/neck or MRA when indicated for large-vessel occlusion/vascular assessment
- ☐ MRI brain with diffusion-weighted imaging when clinically indicated (do not delay time-sensitive reperfusion therapy)
- ☐ Carotid duplex ultrasound if extracranial carotid disease is not adequately characterized by CTA/MRA
- ☐ 12-lead ECG
- ☐ Transthoracic echocardiogram ± bubble study/TEE if cryptogenic or embolic source suspected
- ☐ Extended cardiac rhythm monitoring after discharge if stroke etiology remains undetermined and clinically indicated

8. Medications

8a. Thrombolysis / Reperfusion (if applicable)

- ☐ IV thrombolysis per institutional acute stroke protocol: alteplase or tenecteplase for eligible patients; use actual measured weight and document agent, dose, and treatment time
- ☐ After IV thrombolysis: avoid antiplatelet/anticoagulant therapy for the first 24 h; obtain follow-up brain imaging before initiation, per protocol
- ☐ Endovascular thrombectomy: activate Neurointerventional/CSC pathway for eligible LVO patients, including selected extended-window patients per current protocol

8b. Antithrombotic / Anticoagulation Therapy

- ☐ Aspirin 160-325 mg PO/PR/NG within 24-48 h after onset if no IV thrombolysis, or after 24-h post-thrombolysis imaging excludes hemorrhage; maintenance dose per secondary-prevention plan
- ☐ Dual antiplatelet therapy for eligible minor/nondisabling ischemic stroke or high-risk TIA only per institutional DAPT protocol; specify regimen and duration
- ☐ Oral anticoagulation for atrial fibrillation/flutter or another confirmed cardioembolic source when indicated: agent and start timing individualized to infarct size, hemorrhagic-transformation risk, repeat imaging, renal function, and bleeding risk
- ☐ Routine urgent full-dose heparin is NOT a default treatment for acute ischemic stroke; use only for a separate physician-specified indication per institutional anticoagulation protocol

8c. Blood Pressure Management

- ☐ No IV thrombolysis/EVT and no other hypertensive emergency: generally avoid acute BP lowering unless BP >220/120 mmHg; if treatment is required, individualize and avoid rapid excessive reduction
- ☐ IV thrombolysis: BP must be <185/110 mmHg before treatment and maintained <180/105 mmHg for the first 24 h after treatment, per protocol
- ☐ After EVT: follow the institutional post-thrombectomy BP target based on reperfusion status; avoid intensive routine SBP lowering to <140 mmHg unless another clinical indication exists
- ☐ Antihypertensive agent/route: _____ Resume/initiate long-term BP therapy once neurologically and hemodynamically stable, per physician order

8d. Statin / Lipid-Lowering Therapy

- ☐ High-intensity statin unless contraindicated when guideline-indicated (e.g., atorvastatin 40-80 mg or rosuvastatin 20-40 mg); document alternative/reason if not used

8e. Glucose Management

- ☐ Point-of-care glucose q6h (or per institutional protocol); treat persistent hyperglycemia with a reasonable inpatient target such as 140-180 mg/dL; avoid intensive 80-130 mg/dL control in AIS
- ☐ Treat hypoglycemia promptly per hospital protocol (AHA/ASA recommends treatment when glucose <60 mg/dL); avoid iatrogenic hypoglycemia

8f. VTE Prophylaxis

- ☐ Intermittent pneumatic compression/SCDs for immobile patients unless contraindicated - start promptly
- ☐ Consider prophylactic-dose UFH/LMWH in immobile patients when bleeding risk is acceptable and post-thrombolysis/post-procedure hold requirements are satisfied; use institutional VTE protocol

8g. Other Supportive Medications

- ☐ Antipyretic (e.g., acetaminophen) for temperature >38.0°C - maintain normothermia
- ☐ Bowel regimen as indicated
- ☐ Home medications reconciled - continue/hold per physician review
- ☐ Diabetes medications/insulin per institutional inpatient glycemic protocol; avoid overly intensive glucose lowering and prevent hypoglycemia
- ☐ Avoid medications not indicated for acute ischemic stroke (e.g., nimodipine unless treating aneurysmal subarachnoid hemorrhage)

9. Consults & Multidisciplinary Care

- ☐ Neurology / Stroke Service
- ☐ Stroke Coordinator / Stroke Program notification per institutional workflow
- ☐ Physical Therapy
- ☐ Occupational Therapy
- ☐ Speech-Language Pathology (swallow and communication evaluation)
- ☐ Case Management / Social Work - discharge planning and rehabilitation placement
- ☐ Dietitian if dysphagia, malnutrition risk, or diabetes management is needed
- ☐ Pharmacy - medication reconciliation and anticoagulation dosing/support
- ☐ Palliative Care if goals-of-care discussion is indicated
- ☐ Neurosurgery if hemorrhagic conversion, hydrocephalus, mass effect, or malignant edema risk

10. Patient & Caregiver Education / Discharge Readiness

- ☐ Stroke warning signs (F.A.S.T./B.E.F.A.S.T.) and emergency activation instructions documented and reinforced
- ☐ Individual risk-factor modification (BP, diabetes, cholesterol, smoking, activity, diet)
- ☐ Purpose, dose, and adherence requirements for discharge medications
- ☐ Follow-up appointment(s) scheduled: Neurology/Stroke clinic, primary care, rehabilitation as applicable
- ☐ Written stroke discharge instructions provided
- ☐ Discharge functional status (e.g., mRS) documented and secondary stroke-prevention plan completed

11. Institutional Document Control / Approval

Effective Date	Review Date	Version	Policy/Order Set ID
		2026.1	
Required institutional approvals before clinical activation: Stroke Medical Director / Neurology; Pharmacy & Therapeutics; Nursing; Quality & Patient Safety; and Medical Staff/Hospital governance according to KFCH document-control policy.			

12. Ordering Physician Authorization

Physician Signature: _____ Date/Time: _____
 Print Name: _____ Pager/Contact: _____

References: Prabhakaran S, et al. 2026 Guideline for the Early Management of Patients With Acute Ischemic Stroke: A Guideline From the American Heart Association/American Stroke Association. *Stroke*. Published online January 26, 2026. doi:10.1161/STR.0000000000000513; American Heart Association Get With The Guidelines®-Stroke program resources; applicable KFCH thrombolysis, thrombectomy, antithrombotic, VTE, dysphagia, and inpatient glycemic-management protocols.

Clinical-use statement: This order set is a guideline-aligned institutional template and is not valid for patient care until formally approved and activated through KFCH governance, pharmacy, nursing, quality, and medical-staff processes.